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NOVENA SURGICAL AND ORTHOPAEDIC CENTRE
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--- DISCHARGE SUMMARY ---

PATIENT DEMOGRAPHICS

| PATIENT NAME | AHMAD ZULKIFLI BIN HASSAN
| NRIC | S94XXX7G
| DATE OF BIRTH | 1994-11-22 (30 Years)
| SEX | Male
| MRN | 880251-09
| CONTACT | +65 9123 4567

ADMISSION & DISCHARGE DETAILS

| DATE OF ADMISSION | 2025-05-02
| TIME OF ADMISSION | 11:45
| DATE OF DISCHARGE | 2025-05-04
| TIME OF DISCHARGE | 14:00
| ADMITTING PHYSICIAN | Dr. Tan Boon Kiat
| PRIMARY SURGEON | Dr. Tan Boon Kiat (SMC: MCR-44012D)
| DISCHARGE TO | Home

PRINCIPAL DIAGNOSIS:

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ICD-10	DESCRIPTION
K35.80	Unspecified acute appendicitis

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PROCEDURES PERFORMED:

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CPT CODE	DESCRIPTION
44950	Appendectomy
99232	Subsequent hospital care, per day

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| DATE OF PROCEDURE | 2025-05-02
| ANESTHESIA | General Anesthesia

HISTORY OF PRESENTING ILLNESS:

Mr. Ahmad Zulkifli, a 30-year-old male with no significant past medical history, presented to the A&E on 2025-05-02 with a 24-hour history of abdominal pain. The pain was initially periumbilical before localizing to the right lower quadrant (RLQ). Pain was sharp, constant, and rated 8/10. Associated with nausea, anorexia, and a subjective fever. No vomiting or change in bowel habits.

PHYSICAL EXAMINATION ON ADMISSION:

Vitals: T 38.1 C, BP 130/85 mmHg, HR 98 bpm, RR 18/min, SpO2 99% on room air.
Abdomen: Soft, non-distended. Marked tenderness and guarding over McBurney's point in the RLQ. Positive rebound tenderness. Bowel sounds were present but hypoactive.

DIAGNOSTIC WORKUP:

- WBC: 14.5 x 10^9/L (Elevated) with neutrophilic predominance.
- CRP: 68 mg/L (Elevated).

- Abdominal Ultrasound: Revealed a non-compressible, dilated appendix (diameter 8mm) with surrounding fluid, consistent with acute appendicitis.

HOSPITAL COURSE:

The patient was admitted under Dr. Tan Boon Kiat for surgical management. After obtaining informed consent, he was taken to the operating theatre on 2025-05-02 for an urgent laparoscopic appendectomy.

The procedure was performed under general anesthesia and was uneventful. A severely inflamed, non-perforated appendix was identified and removed. The abdominal cavity was irrigated, and the specimen was sent for histopathology.

Post-operatively, the patient was monitored in the recovery ward and subsequently transferred to the inpatient ward. He was managed with IV fluids, IV antibiotics (Cefazolin and Metronidazole), and IV analgesia. He made a good post-operative recovery. By post-op day 1, he was able to tolerate a soft diet and was ambulating. Pain was well-controlled with oral analgesics. IV line was removed.

On post-op day 2, the patient remained afebrile and hemodynamically stable. His pain was minimal, and he was tolerating a normal diet. The surgical wounds were clean, dry, and intact. A decision was made for discharge.

CONDITION AT DISCHARGE:

Stable, afebrile, and comfortable. Ambulating independently. Tolerating diet and fluids well. Abdomen soft, with minimal tenderness at port sites. Wounds are clean with no signs of infection.

DISCHARGE MEDICATIONS:

1. Arcoxia (Etoricoxib) 90mg - One tablet daily for 5 days (for pain/inflammation)
2. Panadol (Paracetamol) 500mg - Two tablets every 6 hours as needed for pain
3. Augmentin (Amoxicillin/clavulanate) 625mg - One tablet twice a day for 5 days

DISCHARGE INSTRUCTIONS & FOLLOW-UP PLAN:

1. DIET: Resume normal diet as tolerated.
2. ACTIVITY: Avoid strenuous activity and heavy lifting (>5kg) for 4 weeks. May resume light activities and office work after 1 week.
3. WOUND CARE: Keep surgical dressings dry and intact for 3 days. After 3 days, may remove dressings and shower. Do not soak in a bathtub or swim for 2 weeks. Pat wounds dry gently.
4. MEDICATION: Complete the full course of prescribed antibiotics. Take painkillers as needed.
5. URGENT REVIEW: Return to the A&E if you experience high fever (>38.5 C), increasing abdominal pain, vomiting, redness, swelling, or discharge from the wound sites.
6. FOLLOW-UP: Scheduled for a post-operative review at the Novena Surgical and Orthopaedic Centre clinic in 2 weeks.

Date: 2025-05-19 (Monday)

Time: 10:30 AM

This summary was electronically generated and verified.

Electronically Signed,

Dr. Tan Boon Kiat
Consultant General Surgeon
Novena Surgical and Orthopaedic Centre

Date of Dictation: 2025-05-04

Date of Transcription: 2025-05-04

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